

CENTERS FOR MEDICARE & MEDICAID SERVICES

Medicare Learning Network® (MLN)

Remote Patient Monitoring (RPM) — Billing & Coverage Policy

Document Version: CY 2024–2025 (Post-PHE Final Rule)

Effective Date: January 1, 2024 – December 31, 2025

Source: Medicare Physician Fee Schedule Final Rule / MLN Booklet MLN901705

1. Overview

Remote Patient Monitoring (RPM) allows a patient to collect their own health data using a connected medical device that automatically transmits data to their provider. The provider uses this data to treat or manage the patient's condition. This document defines the coverage criteria, billing requirements, and applicable CPT codes for Medicare reimbursement of RPM services.

2. Coverage Rules

Rule R001 — Patient Eligibility — Established Patient Requirement

Effective January 1, 2024, RPM services may only be furnished to ESTABLISHED patients. An established patient is one who has received professional services from the billing practitioner, or another practitioner of the same specialty in the same group practice, within the last 3 years. The COVID-19 PHE waiver permitting new patient enrollment has expired. Exception: patients who initiated RPM during the PHE are grandfathered as established patients. A new patient must complete an Evaluation and Management (E/M) visit before RPM may be initiated.

Applicable CPT Codes: 99453, 99454, 99457, 99458

Rule R002 — Minimum Data Collection Days

To bill CPT codes 99453 and 99454, the practitioner must collect physiologic data for a minimum of 16 days in a 30-day billing period. The COVID-19 PHE exception allowing 2-day minimum collection has expired for all patient types. The 16-day requirement applies uniformly across all RPM-eligible conditions. Note: The 16-day requirement does not apply to treatment management codes 99457 and 99458.

Applicable CPT Codes: 99453, 99454

Rule R003 — Billing Frequency — One Practitioner Per Period

Only one practitioner may bill RPM codes per patient during a 30-day period. Even when multiple medical devices are in use, RPM services are billed only once per patient per 30-day period. This policy is unchanged from prior periods.

Applicable CPT Codes: 99454, 99457, 99458

Rule R004 — RPM and RTM Concurrent Billing

RPM (remote physiologic monitoring) and RTM (remote therapeutic monitoring) may NOT be billed together for the same patient in the same 30-day period. This restriction is unchanged. Practitioners must choose one monitoring type per billing period.

Applicable CPT Codes: 99457, 99458

Rule R005 — Rural Health Clinics (RHCs) and FQHCs — Billing Eligibility

Effective January 1, 2024, Rural Health Clinics (RHCs) and Federally Qualified Health Centers (FQHCs) ARE now eligible to bill for RPM services under HCPCS code G0511 (General Care Management). Eligibility requires a minimum of 20 minutes of qualifying non-face-to-face RPM services furnished during a calendar month. This represents a significant expansion of RPM access in underserved rural and community health settings.

Applicable CPT Codes: G0511

3. Applicable CPT Codes — Quick Reference

CPT Code	Description	Notes
99453	RPM setup and patient education	One-time per episode
99454	RPM device supply and data transmission	Per 30-day period
99457	RPM treatment management, first 20 min	Per 30-day period
99458	RPM treatment management, addl 20 min	Per 30-day period

4. Disclaimer

This document is derived from CMS Medicare Physician Fee Schedule Final Rules and MLN publications. Content has been structured for research and demonstration purposes. Always refer to the official CMS Medicare Coverage Database (MCD) and the applicable Final Rule for authoritative guidance.